Journal of Clinical Epidemiology 198 (2026) 112394

ORIGINAL RESEARCH

A scoping review of guidance on sharing trial results with patients and the public finds inconsistent recommendations and limited methodological development

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Accepted 22 June 2026; Published online 26 June 2026

Abstract

Objectives: To identify, analyze and summarize existing guidance on sharing trial results with patients and the public and to provide recommendations for developing a high-quality reporting guideline that is user-friendly, effective, and accessible.

Study Design and Setting: We included English-language documents providing guidance or recommendations on sharing research study results with patients and/or the public, published anytime, in journals or gray literature. We systematically searched Ovid MEDLINE® and Web of Science Core Collection, performed citation tracking in PubMed®, and conducted extensive gray literature searches, including Google and Google Scholar, and other key websites (eg, EQUATOR Network, government and research organizations, publishing groups, research and policy centers). Titles and abstracts were screened for eligibility, followed by full-text review. The results were thematically analyzed.

Results: A total of 2289 potentially eligible records were screened, and 24 met eligibility criteria and included in the review. Of these, only 9 were broadly applicable to clinical trial designs. No specific guidance or reporting guideline was identified focusing on sharing results of randomized trials with patients and the public. Recommendations on content, presentation, timelines, and dissemination varied in quality and specificity. Most guidance documents lacked details on tailoring information for diverse audiences, with minimal focus on cultural sensitivity or inclusivity. Only five of the documents had been pilot-tested prior to implementation, and stakeholder involvement was reported in 13, though often inconsistently.

Conclusion: Althougb there has been progress in developing guidance for sharing research results, many existing documents lack a standardized approach, do not sufficiently address diversity considerations and have not undergone formal evaluation. Future efforts should focus on coproducing a tailored reporting guideline following established methodology with meaningful stakeholder-involvement and pilot-testing to ensure that recommendations meet the diverse needs of patients and the public. © 2026 The Author(s). Published by Elsevier Inc. This is an open access article under the CC BY license (http://creativecommons.org/licenses/by/4.0/).

Keywords: Randomized controlled trials; Patient and public involvement; Research result dissemination; Lay summaries; Research transparency; Reporting guidelines

Funding: This scoping review constitutes an integral component of Sanjana Choudhury's DPhil project which is funded by a DPh studentship jointly covered by the Clarendon Fund and a Nuffield Department of Orthopedics, Rheumatology and Musculoskeletal Sciences Studentship by the University of Oxford.

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https://doi.org/10.1016/j.jclinepi.2026.112394

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Plain Language Summary

Sharing results in a clear, respectful way is an important part of doing good research. Evidence suggests that people who take part in research are routinely not told what the study found. However, there is little guidance available for researchers on how to do this in a meaningful way. In this review, we looked for any existing guidance on how to share research results with patients, and the public. We looked for both published and unpublished guidance documents. We found 24 documents, some of them had useful suggestions, but many were hard to find, vague, or missing important details. Many of them did not include input from patients or the public. None were made specifically for sharing results from randomized trials. Although there is some guidance available, there are still significant gaps. Our findings show that researchers need better and clearer guidance on how to share trial results. This would help make the information easier to understand, more inclusive, and more helpful for both patients and the public.

1. Introduction

Under the new UK clinical trials regulations, effective 28 April 2026, sponsors must provide participants with a plain language summary of trial results within 12 months of trial completion [1]. The regulations reflect a growing emphasis on transparency and the need for guidance on effectively sharing trial results with participants.

Researchers and participants alike value the opportunity to share and receive trial results [2,3]. However, evidence shows that in practice results are rarely shared with participants. According to the UK Health Research Authority's 2021 annual report, 90% of clinical trials had not shared the findings with their participants [4]. A survey published in 2003 found that only five out of 150 institutions had established mechanisms to share study results with participants [5]. More recent data published in 2019 showed that out of 1818 trials, only 27% reported sharing trial results with participants, 13% planned to offer results, 10% were unsure and 33% never planned to share results [6]. Getz and Farides-Mitchell reported that only about 2% of clinical trials worldwide completed between 2015 and 2017 shared results with study participants [7]. Even when participants received lay summaries, one study found that over 85% were ''difficult'' to read [8].

Studies have identified several barriers to effective trial result reporting. Research shows that uncertainty among researchers about what information to share, how to present it, and when to disseminate findings, along with a lack of institutional systems or clear guidance, are major obstacles [6,9−13]. Reporting guidelines which provide recommendations on what information should be reported when sharing trial results exist for scientific audiences (eg, the CONSORT Statement). However, there is a lack of a high-quality reporting guideline with key recommendations on what should be reported when sharing trial results with patients and the public [14]. A reporting guideline for sharing trial results with patients and the public will help address these uncertainties and provide a structured framework for researchers planning to share such information. Developing such a guideline requires a rigorous, multistage process, beginning with a comprehensive review of existing resources [15].

The objective of this scoping review was therefore to identify, describe, and summarize existing guidance on sharing trial results with patients and the public. Broader study types were also considered to capture all relevant approaches to communicating research results with patients and the public, given the anticipated paucity of trial-specific resources. We aimed to map current practices, identify gaps, assess the need for a reporting guideline, and inform its key components for sharing trial results with patients and the public. In addition to identifying core components of reporting guidance, this review considered how existing recommendations address accessibility, inclusiveness, and the needs of diverse populations. These factors are central to ensuring that communication of trial results is meaningful, equitable, and relevant to all patient and public audiences.

2. Methods

2.1. Protocol and registration

The review followed a protocol that was agreed within the team prior to screening and data charting, which was retrospectively uploaded to OSF (15 July 2024) to make it publicly accessible. The search was conducted on 12 March 2024 [16]. No eligibility criteria or outcomes were changed after the search was run.

We have reported the review in accordance with the Preferred Reporting Items for Systematic reviews and Meta-Analyses extension for Scoping Reviews (Appendix A) [17].

2.2. Terminology

In this review, the terms patients and participants are used interchangeably. Although the term ''participant'' might be more accurate to describe individuals involved in a study

What is new?

Key findings

- We identified 24 guidance documents focused on sharing study results with patients and the public. Nine were broadly applicable to clinical trial designs. No specific guidance or reporting guideline was identified for sharing results of randomized trials with patients and the public.
- Existing guidance is scattered across sources and often embedded within broader dissemination frameworks, making it difficult to locate or apply.
- Only a few reported diversity considerations or stakeholder engagement in their development. And only five of the guidance documents reported pilot-testing prior to publication

What this adds to what is known?

- This is the first scoping review to map and appraise existing guidance on sharing trial results with patients and the public from a reporting perspective.
- The findings highlight the lack of a structured, evidence-based reporting guideline specifically for sharing results of randomized trials with patients and the public.

What is the implication and what should change now?

- Our findings support the development of a dedicated reporting guideline to help researchers ethically and effectively share the results of randomized trials with patients and the public.
- Guideline development should prioritize clarity, accessibility, coproduction with patients and the public and adaptability to different contexts and populations.

or trial, we chose the term ''patients'' due to its widespread usage in the ''patient and public'' terminology [18,19].

2.3. Eligibility criteria

Eligibility criteria were defined using the Participant-Concept-Context framework.

Participants: Patients, trial participants, and the public.

Concept: Guidance or recommendations on sharing study or trial results.

Context: No restrictions on setting.

We included documents providing guidance on sharing study or trial results with patients or the public, regardless of publication type (eg, journal articles, organizational reports, gray literature), written in English and relating to human studies.

We excluded guidance aimed solely at researchers (eg, CONSORT) [14], documents that did not specify the content to be shared, general commentary on dissemination or plain language writing, and guidance limited to individual clinical results.

2.4. Search strategy

A multistage search strategy was developed, involving a systematic database search (Stage 1), citation tracking (Stage 2), and an extensive gray literature search (Stage 3). Full details of the search strategies are reported in Appendix B.

2.4.1. Stage 1: systematic search

Two bibliographic databases were searched to identify relevant articles. First, we searched Ovid MEDLINE® from 1946 to 12 March 2024. Second, we searched the Web of Science Core Collection, covering records from 1900 to 12 March 2024 depending on the specific index. Both searches were conducted on 12 March 2024 with no date limits applied. The search strategies for both databases were reviewed by two information specialists who reviewed the structure, terminology, and syntax of the search queries (Available in Appendix B). At this stage, 1303 records were screened, and 18 records met the eligibility criteria for inclusion in this review.

2.4.2. Stage 2. citation tracking

Using the 18 articles identified in stage 1 as a starting point, additional relevant articles were explored in PubMed® between 13 April 2024 and 23 May 2024. For each included article, we reidentified the record in PubMed®, screened reference lists (backward snowballing) and ''Cited by'' lists (forward snowballing). We also used PubMed's ''Similar articles'' feature to identify any further studies. At each step, the first 10 results were screened, yielding an average of 30 additional records for each article retrieved in stage 1. Duplicates already identified in the initial search were excluded from further screening.

2.4.3. Stage 3. gray literature search

An extensive gray literature search was performed to complement the database searches, conducted in three parts. First, a Google Scholar search was carried out on 23 May, 30 May, and 12 June 2024 using combinations of free-text search terms to ensure maximum coverage (Appendix B). Search results were sorted by relevance, and the first 10 pages (~100 records per search) were screened to identify relevant documents. This number of pages was chosen to identify many of the most relevant hits while maintaining a manageable amount to screen.

Second, a Google search was performed on 15 June 2024 using keywords such as

"guidance + results + patients + public," and the first 100 results were retained for further screening (Appendix B).

Third, relevant organizational websites were hand-searched using the site's search function and combinations of keywords (eg, "guidance", "results", "patients", and "public"). Websites were identified through the database and gray literature searches, as well as the authors' existing knowledge. The websites included government organizations (eg, Health Research Authority), grant funding bodies (eg, NIHR), research and policy centers (eg, Multi-Regional Clinical Trials Center of Brigham and Women's Hospital and Harvard), charity organizations (eg, Cancer Research UK), the EQUATOR Network, publishing groups (eg, Taylor & Francis), patient participation groups (eg, OPENARMS), and workgroups of medical journal editors. (A full list is provided in OSF) [16].

Note: Following peer review, Google Scholar searches were rerun using revised syntax and we screened the first 100 results sorted by relevance. No additional eligible guidance documents were identified beyond those already included in the review.

2.5. Selection of documents for inclusion

All records were imported into EndNote (v21), where duplicates were removed and screening was conducted. Screening was performed in two stages: (1) title/abstract screening and (2) full-text screening. Title/abstract screening was conducted by one reviewer. Full-text screening involved a second reviewer, who independently verified the inclusion of records, with disagreements resolved through discussion and consensus. Reasons for exclusion at the full-text stage are reported in the PRISMA-ScR flow diagram.

2.6. Data extraction

A data charting form was developed and piloted on the first five included documents by two authors (SC and SH). The form was refined based on this pilot, after which data were charted by one reviewer using EndNote and extracted into a structured form using Microsoft Excel (Appendix C). Any uncertainties were discussed with a second reviewer. Any disagreements were resolved by discussion and consensus with the wider team.

2.7. Data analysis and presentation of results

Data were collated, summarized, and analyzed according to key features of each guidance document. Data were categorized into the following themes: characteristics of the guidance, recommendations on content and presentation, patient and public involvement in development of the guidance, timeline and dissemination of results, and the development methods and, where applicable, evaluation of the guidance documents.

2.8. Data quality

The PRISMA-ScR statement notes that critical appraisal is not a required component of scoping reviews, and currently, there are no recognized tools for evaluating the quality of reporting guidelines [17].

However, to assess the rigor and relevance of existing guidance documents, we reviewed the following key domains relevant to reporting practices:

- **Scope and purpose:** clarity of aims and target audience
- **Development methods:** whether systematic processes (eg, literature reviews, expert consultation) supported the development
- **Stakeholder involvement and coproduction:** involvement of patients, public, or participants in development
- **Content:** aspects of results reporting covered (eg, individual vs summary results, format, "what" to share, timing and dissemination strategies)
- **Accessibility:** availability, clarity, and usability for nonspecialist audiences

To improve transparency, each domain was assessed using a simple coding framework:

- ***reported*** *(clearly described in the document),*
- ***unclear*** *(mentioned but lacking sufficient detail), or*
- ***not reported*** *(no information provided)*

The full table is provided in Appendix H.

3. Results

3.1. Search results

We identified 2289 potentially eligible records (database searches: 1703, citation tracking: 486, gray literature: 100). After title and abstract screening, 162 records were assessed at full text, of which 24 met the eligibility criteria and were included. The search and flow of the inclusion process are described using the PRISMA flow diagram and shown in Figure [17].

3.2. Characteristics of the included guidance documents for sharing study results

Of the 24 included guidance documents, most (10/24) applied broadly to any type of research study, while 9 focused on different clinical trial designs [21−29]. A small number of documents addressed systematic or evidence reviews [2], biospecimen studies [1], biomonitoring studies [1], communication research [1]. None were specifically designed for reporting results of randomized trials.

The 24 guidance documents were mostly published in North America and Europe, with an increase in documents observed over the last decade. Eight documents were peer-

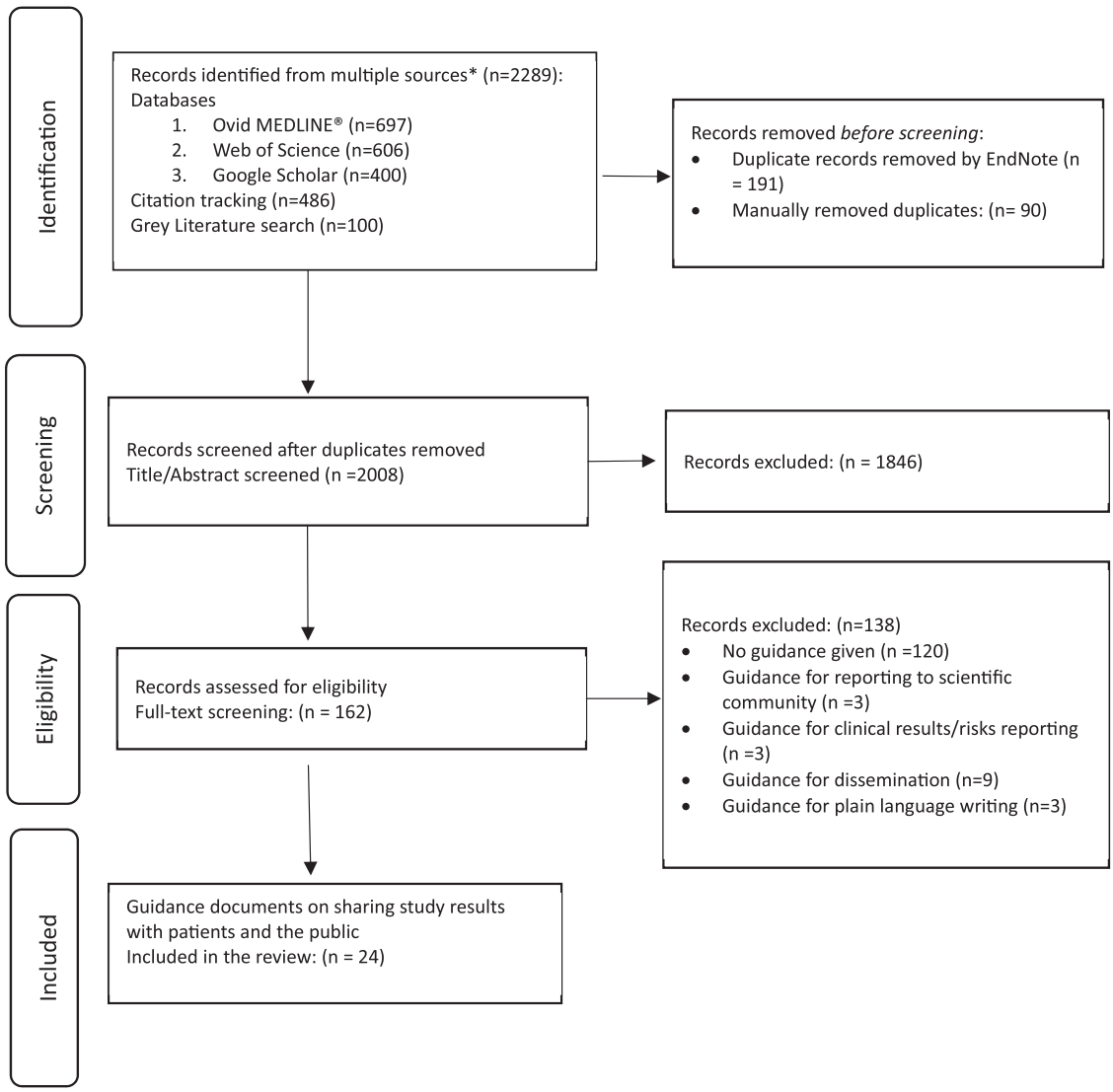


Figure. Flow chart of screened and included documents (adapted from PRISMA 2020 flow diagram for systematic reviews [20].

reviewed; others were published on websites of government bodies, research organizations, regulatory agencies, charities, publishers, or industry. Most documents address sharing summary results, two focus on individual participant results, and one covers both individual and summary level [30–32].

One report is under review by the United States Food and Drug Administration (U.S. FDA) and not officially published yet (proposed guidance under 21 CFR 10.115(f) [3]. [25] A full summary of characteristics is provided in Appendix D.

3.3. Suggested content for sharing study results

The 24 guidance documents offered a number of recommendations on the content for sharing study results with patients and the public. Table 1 summarizes these recommendations, highlighting the varying levels of detail provided across the included document. Appendix E outlines specific recommendations provided by each individual guidance document.

3.4. What are the recommendations on presenting the study results?

Of the 24 guidance documents, 23 documents provided recommendations on how to present the results to patients and/or the public. Most highlighted the importance of using plain language and to avoid the use of jargon. Few emphasized the need to consider cultural differences and sociodemographic characteristics. A summary is provided in Table 2, with more detailed tables in Appendix F.

3.5. Patient and public involvement (PPI)

Thirteen of the twenty-four guidance documents reported incorporating patient and public involvement to some extent during the development stage (eg, discussions, consultations, or public workshops). Appendix D outlines if PPI input was taken during development of each guidance.

Fourteen of the 24 guidance documents explicitly advocated for incorporating PPI at various stages of the result-sharing process ranging from input in the initial planning to final review and dissemination.

3.6. What are the recommendations for the timeline and dissemination of study results?

Of the 24 guidance documents included, 10 provided recommendations regarding the timing of sharing study results with patients and the public. Sharing results within 1 year of completing primary outcome data collection was a common recommendation, with shorter or longer timelines advised for certain study types (eg, 6 months for pediatric trials) [23–25,34].

Documents also recommended involving ethics boards and patient partners [21], clarifying timing in consent forms [41], and sharing results only after peer review [30].

Thirteen documents offered varying recommendations on dissemination strategies, including making results publicly accessible (eg, open access or public websites) [22,27,28,32,34,35,44] and using multiple formats such as digital platforms, printed materials, or in-person sessions [30,41].

Twelve documents provided toolkits or practical resources (eg, checklists, templates) to support dissemination [21–27,33,35–37,39,40]. Seven documents recommended

Table 1. Suggested content for sharing results with patients and the public

Category	Key content items [reference #][a]
Thank you	To study participants, sites, trial teams, and any other relevant contributors [21,22,24–28,33–37]
Introduction	General description of the risks and benefits of receiving results [34,38]
Background	Primary goal, aims, objectives, research question [21,23–26,29,30,32–34,36–43]
General study information	- Title/trial name (formal & lay), protocol number, investigators, sponsor, funder, study dates, trial registration ID - About the trial: [eg, disease, intervention, investigational products used, study phase/design] - Optional diagram of protocol flow [21–27,29,33–35,38–41,43]
Study methods	- Major interventions, such as randomization - Any issues requiring changes to the trial [21,22,27,29,34–37,39,40,42,44]
Study participants	- Participant characteristics, inclusion/exclusion criteria, numbers overall and per arm - A clear statement that the results apply to the study population, and not any other population, and under these specific conditions [21–27,33,35,36,42,44].
Key results/outcome	- Primary and secondary outcomes - Short/long-term treatment effects - Interpretation whether positive/negative findings - Implications for practice/research, uncertainties - A statement that the summary includes the results of a single study and that multiple studies are used together to assess the risks and benefits [All documents]
Side effects	- Key, serious or unexpected side effects - Long-term side effects and follow-up recommendations (if any) [21–27,29,30,35,40,41]
Next steps	- Follow-up studies, future research, potential intervention availability - Next steps (eg, publication, licensing) [21–27,30,32–36,39]
Patient engagement and community involvement	Patient and public involvement and engagement (PPIE) in the design or the study delivery [21,27,33,35,36,42]
Trial conclusions	Conclusions and interpretations [21,33]
Shared decision-making prompt	- How to discuss trial results with personal health care providers - A statement that patients should consult their physicians or study doctors with further questions about their individual care and should not make changes in the treatments based on the results of this study [21,22,25,28,33]
Trial limitations	Excluded populations, evidence limitations etc. [21,22,30,40]
Date	Date of summary, updates, peer review completion [24–27,30,33,34,40]
Contact information	Sponsor contact details [25,33]
Further information	Links to publications, study website, institutional contacts [21–30,32–37,45]
QR code	Link to digital version of lay summary, if applicable [33]
Disclosures	Drug approval status, authorship/funding transparency, data protection policy [30,33]

[a] [Reference # (Report ID which contains the recommendation].

Table 2. Recommendations for presentation of study results

Category	What are the recommendations on presenting study results?
Text structure and style	- Use text hierarchy, simple headings/subheadings, and bullet points [21,23,24,26,29,31,33,40] - Apply a ''Funnel'' structure: (inverted pyramid writing style) [23,26,33,42] - Keep font readable (≥12 pt), sentences and paragraphs short [23,24,26,29,33,35,36,38,40,44] - Open question headers: to engage readers [33] - Use active voice, simple nontechnical language [21–24,26,29,36,38–40,44] - Use respectful, culturally sensitive language, provide relevant examples [23,24,32,33,35,38–40] - Use bold for emphasis, avoid underlining, italics, fancy fonts, all CAPS [23,24,26] - Use pronouns. Write in the first-person plural [40].
Numeracy and data representation	- Use natural frequencies and percentages; avoid statistical terms [23,24,26,33,40]. - Avoiding superlative and enthusiastic words. Use neutral consistent language [24,25,28,35,44]. - Prefer real images or bespoke illustrations over stock photos [42] - Ensure clear graph labels and meaningful scales [24].
Design and layout	- Separate sections with color blocks, divider lines, and white space [23,24,26,33,40,42] - Ensure text/background contrast, use left justified text [24]. - Share electronically as a PDF [21,22]. - Avoid using blue font for regular text to avoid confusion with hyperlinks [21].
Engagement and accessibility	- Involve patients, public, and plain-language professionals in development/review [26,36,39,40,42] - Use accessible level of English and translate if relevant [32,34] - Write for international audiences; avoid regional terms (like ''A&E'' or ''ER'') [40]. - Use graphics, visuals, or videos (with closed captions) [21–24,26,29,31–33,35,42]
Review and quality control	- Read the summary aloud to catch errors or awkward expressions [39] - Ensure grammar, punctuation and spelling are accurate [38] - Sentences should be phrased positively, rather than negatively [38] - Provide a clear, concise title and introduction [38] - Text should be suitable for people with a low to average level of literacy. (~12 y/6th grade/8th grade) [23,24,29–31,36,42,44]. - Minimize hyperlinks and unnecessary imagery; avoid assuming prior knowledge [23,25,29,35,42]

early planning and budgeting for dissemination in research protocols and grant applications [24,28–30,32,41,44].

Only two documents emphasized tailoring dissemination approaches to participants' cultural needs, preferences, logistical constraints, and study population [24,26].

More detailed tables can be found in Appendix G.

3.7. Development of the guidance

The methods used to develop the 24 included guidance documents were often unclear or insufficiently reported. None of them reported adhering to the recommended steps for developing a reporting guideline [15]. For 10 documents, no information on development methods could be identified; it is possible that they undertook similar steps but did not report them. Another 10 documents provided detailed accounts of their development, and four offered little description. Only five documents reported pilot-testing before implementation and just four referenced considerations of diversity. A summary of reported development methods is presented in Table 3.

4. Discussion

4.1. Key summary of findings

To our knowledge, this is the first scoping review to identify existing guidance on sharing trial results with patients and the public. We identified 24 guidance documents, providing valuable insight into current practices and recommendations. Although multiple guidance documents exist, their quality varied, many were not easily accessible, and most did not explicitly describe their development methods. The guidance was often scattered across various sources or embedded within broader dissemination frameworks, limiting visibility for researchers planning to share results. Importantly, none were specifically designed for reporting the results of randomized trials with patients and the public, nor developed as reporting guidelines following established methodology [15].

Our findings revealed considerable variability in the quality and specificity of recommendations regarding their content, presentation, timeline, and dissemination strategies. Many lacked details on what information to share, how to tailor it for diverse audiences and most had little or no focus on cultural sensitivity.

4.2. Challenges in searching

Much of the existing research on sharing results with patients and the public was not published in journal articles but instead on organizational websites such as the guidance published by TransCelerate BioPharma Inc. in 2016 [28]. Even within published literature, inconsistent terminology made it difficult to capture relevant records. Terms such as ''sharing'', ''disseminating'', ''returning'', and

Table 3. Development methods reported by the guidance documents ($n = 14$)

Report ID	Development method	Level of reporting	Reported diversity consideration?	Reported pilot-testing before implementation?
Arnstein et al [33]	Systematic review and Plain Language Summary of Publications workshop	Detailed	Not reported	No
Baba et al [21]	Virtual workshops with pediatric researchers, healthcare professionals, youth and parents	Not detailed	Not reported	Yes
Botkin et al 2018 [32]	Expert committee with public input and workshops, interviews with community representatives and literature reviews	Detailed	Not reported	No
Cochrane 2022 [40]	Built on existing resources and expert input	Not detailed	Not reported	No
Dunagan et al 2013 [31]	Multi-institute collaboration, literature review, interviews, and workshops	Detailed	Yes	Yes
European Commission 2021 [24]	Multi-stakeholder approach with expert consultation and review	Detailed	Not reported	
Fernandez et al 2012 [34]	Literature review and multidisciplinary discussions by a Return of Results Task force	Detailed	Yes	
Hintz & Dean [30]	Systematic literature review and expert opinion synthesis	Detailed	Not reported	
Knowles et al [42]	Think Aloud Interviews and Co-Design Workshop	Not detailed	Not reported	
MRCT Center,2017 [26]	Multistep collaborative approach with diverse workgroup	Detailed	Not reported	Yes
Rosenberg et al 2021 [43].	Expert consultations followed by public discussion	Detailed	Not reported	
Solovyeva, O., et al 2023 [27]	Review of resources, expert discussions, and wider consultations from a formal PPIE working group	Detailed	Not reported	Yes
Vanderbilt Institute for Clinical & Translational Research [36,37]	Focus groups and dissemination study	Not reported explicitly. Their dissemination research was found after further searching [46,47].	Yes	No
Zimmerman 2022 [29]	Collaboration with science communication experts and qualitative study testing lay summaries	Detailed	Yes	Yes

‘‘providing’’ results were often used interchangeably, as were terms such as ‘‘participant’’, ‘‘volunteer’’, ‘‘patient’’, and ‘‘public’’. This variability made it challenging to develop a sensitive search strategy. The ubiquitousness of common keywords like ‘‘results’’, ‘‘reporting’’, or ‘‘patients’’ yielded a large volume of unrelated articles, complicating efforts to refine the search. To address this, we used a multistage broad search strategy: beginning with a systematic database search, followed by citation tracking, and ending with an extensive gray literature search to ensure maximum coverage of potential sources. The lack of standardized terminology in this evolving area is

understandable but presents a barrier to synthesizing evidence. Future work should prioritize agreeing on common terms to improve future searches.

We included the Web of Science database in addition to MEDLINE to ensure broader coverage. Web of Science also includes conference proceedings and gray literature, which can provide additional valuable insights for a scoping review. This approach helped to minimize publication bias and maximize the diversity of perspectives in the review.

Additional challenges emerged in the gray literature phase, particularly with Google searches. Search results can be influenced by user behavior; therefore, the same device was used across all sessions, and each search was conducted in one sitting with all top 100 links reviewed. As noted by Haddaway et al., while such searches may be less precise, they can improve the comprehensiveness of evidence identification when used alongside structured database searches [48].

4.3. Scope of guidelines and lack of specificity

It was difficult to identify guidelines focusing specifically on sharing trial results with patients and the public. Most guidance documents focused on dissemination and communication strategies (eg, the ''how and why'') rather than specific content to share (''what''). They also lacked tailored guidance for randomized trials. This lack of specificity makes it tricky to apply the general guideline suggestions in the context of randomized trials where the nature of the data, the need for blinding (when applicable), and ethical obligations of sharing results are unique and require clear, tailored communication.

Just as the CONSORT statement provides guidance for reporting randomized trials to academic audiences, there is a clear need for similar guidance aimed at reporting results of randomized trials to patients and the public [14].

4.4. Lack of reporting of development methods

The development process for the reviewed guidance documents was often unclear. In 14 of the 24 documents, clear information could not be found on how they were developed and evidence on the core considerations for production and implementation was absent. This limits the transparency and robustness of their recommendations.

Only five reported pilot-testing of the guidance document before implementation which have improved some approaches, such as making the resource more relevant and user-friendly [29]. However, pilot-testing were in most cases limited in scope, with small samples and narrow audiences. Best practice in guideline development recommends robust, iterative testing with diverse end-users to ensure clarity, usability, and impact [49,50].

4.5. Limited stakeholder involvement in development

The lack of stakeholder engagement in the development stages was also notable. While around half of the guidance documents reported PPI input (eg, consultations or workshops), the extent of involvement varied, and few demonstrated evidence of true coproduction [51].

Very few guidance documents referenced diversity considerations, and none sufficiently addressed cultural and linguistic factors or reported considering factors like religion, sexuality, or other cultural differences, despite their importance in sensitive result-sharing practices. Furthermore, most guidelines originate from North America and Europe, with limited strategies for resource-poor regions that face unique challenges due to funding constraints. Future guideline development should involve a broader range of stakeholders to ensure recommendations are culturally sensitive and inclusive.

4.6. Implications for future practice and research

Our findings highlight several areas for improvement in the development and content of guidelines for sharing trial results with patients and the public. Firstly, there is a clear need to establish standard terminology in this topic to ensure future research is more effective. Secondly, the specificity and scope of the guidelines can be improved. A reporting guideline specific to sharing results of randomized trials with patients and the public is essential. Thirdly, future guideline development must prioritize coproduction with stakeholders and engage a diverse group of people to make sure that the recommendations are more relevant, culturally sensitive and understandable to the target groups. Finally and most importantly, guidelines must be pilot-tested to ensure effectiveness in real-world settings prior to their implementation.

Moving forward, comprehensive, targeted, evidence-based reporting guidelines should be developed that are coproduced with stakeholders, rigorously evaluated for effectiveness and easily accessible in literature.

4.7. Limitations

Title/abstract screening and data extraction were primarily conducted by a single reviewer. A second reviewer screened full texts, piloted the data charting form, and reviewed full-text eligibility decisions. This pragmatic approach of partial single and partial double screening and data extraction was undertaken to balance resource limitations with the gold-standard of double screening and data extraction. However, this approach may have reduced the opportunity to identify discrepancies during study selection and data extraction. It may have increased the risk of missed studies, selection bias, or extraction errors.

The review protocol was registered retrospectively, which reduces transparency regarding whether methodological decisions were fully specified before study

identification and selection began. Although no major changes were made to the review objectives or eligibility criteria following registration, this remains a limitation. The review included only English-language documents, which may have led to the exclusion of relevant guidance developed in other languages or settings. Consequently, perspectives from non-English-speaking contexts may be underrepresented.

Additional limitations arose during the gray literature search. Google Scholar was used as a supplementary source to identify potentially relevant guidance documents; however, the platform's search functionality is not fully transparent or reproducible, and search results can be influenced by syntax, spacing, and relevance ranking algorithms. Although searches were revised and rerun following peer review and no additional eligible documents were identified, some relevant guidance may not have been captured. Future reviews may benefit from further methodological refinement and optimization of search syntax specifically tailored to gray literature platforms.

Despite these limitations, the review provides the first structured overview of existing guidance on sharing trial results with patients and the public. The aim of the review was to map available guidance, identify areas of agreement and inconsistency, and inform the development of a reporting guideline. Although the methodology chosen may have risked missing some eligible items, it is unlikely that additional items would have greatly swayed the overall findings, considering the breadth of what was found and their broad agreement. Although the findings should not be interpreted as a definitive account of all available guidance, they do provide a useful foundation for future research and guideline development in this area.

Ethics statement

Since this is a scoping review, the information retrieved was preexisting and already published. Hence, ethical permissions are not required.

CRediT authorship contribution statement

Sanjana Choudhury: Writing − review & editing, Writing − original draft, Visualization, Validation, Software, Resources, Project administration, Methodology, Investigation, Funding acquisition, Formal analysis, Data curation, Conceptualization. **Gary S. Collins:** Writing − review & editing, Supervision, Conceptualization. **Charlotte Albury:** Writing − review & editing, Supervision. **Jennifer de Beyer:** Writing − review & editing, Supervision, Conceptualization. **Sally Hopewell:** Writing − review & editing, Validation, Supervision, Methodology, Conceptualization.

Declaration of competing interest

There is no conflict of interest in this article. G. S. C. is a National Institute for Health and Care Research (NIHR) Senior Investigator. The views expressed in this article are those of the author(s) and not necessarily those of the NIHR, or the Department of Health and Social Care.

Acknowledgments

We thank Shona Kirtley and Kat Steiner for reviewing and providing feedback on the systematic database search strategy. G. S. C is a National Institute for Health and Care Research (NIHR) Senior Investigator. The views expressed in this article are those of the author(s) and not necessarily those of the NIHR, or the Department of Health and Social Care.

Supplementary data

Supplementary data related to this article can be found at https://doi.org/10.1016/j.jclinepi.2026.112394.

Data availability

Detailed data tables analysed during this study are available in the Appendices.

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